State Spending on Community Living for People with Disabilities

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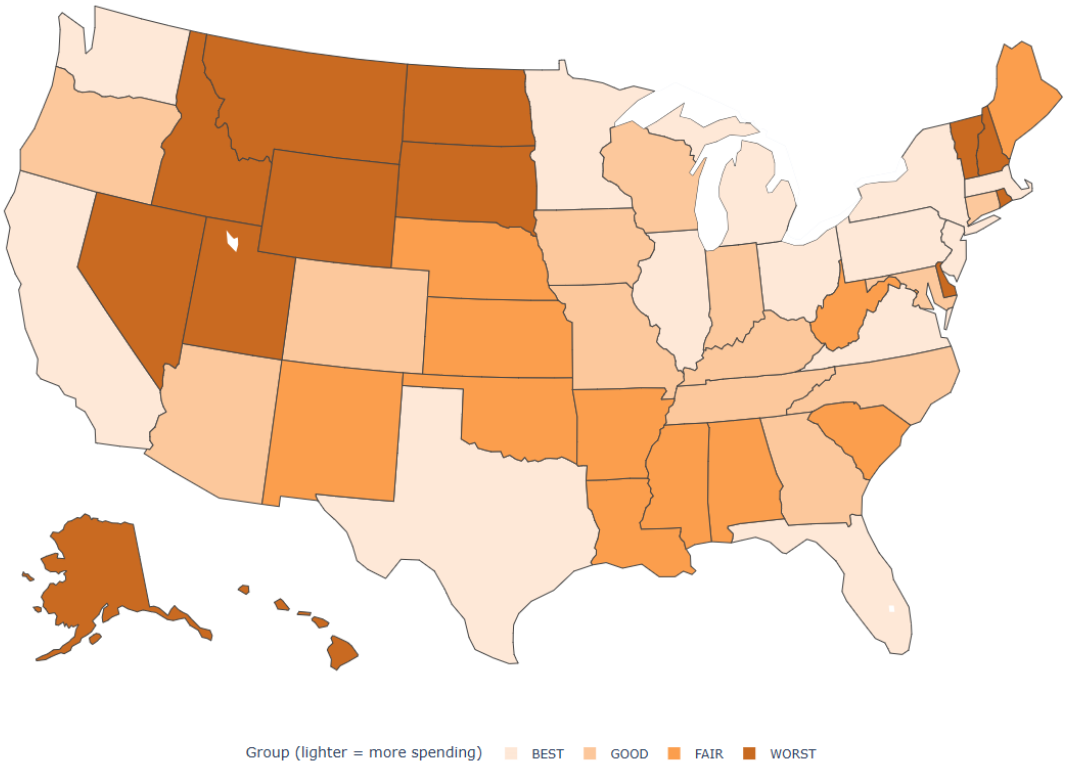
The Americans with Disabilities Act (ADA) mandates the right to live in the community with supports instead of living in a nursing home or long-term care institution. This right was upheld by the U.S. Supreme Court in the landmark Olmstead decision in 1999.[1]

Despite this right to live in the community, some states are doing worse on overall community spending, and some are doing better to support people with disabilities to live in the community. The map and tables below display total state spending per Medicaid recipient, inclusive of all waiver types (Community First Choice, Section 1915 (i), Section 1915 (c), Section 1515), for people with disabilities (all ages) living in the community.[2] Lower spending (represented by a darker color) means fewer financial resources for people with disabilities to live in the community (or more spent on institutions like nursing homes), while higher spending (light colors) means better financial support for people with disabilities to live in the community. Rebalancing money toward community living with support remains an ADA-related disparity issue for many people with disabilities, putting them at higher risk for institutionalization, which also carries a higher risk for COVID-19. Currently, this community living support is highly dependent on your zip code and the state in which you live.

Map: Total Annual Community Living Spending per Medicaid Recipient among People with Disabilities (PWD), all ages, by state



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Data for Puerto Rico, the Virgin Islands, and Guam is unavailable

To explore the interactive version of this map, visit the interactive color-coded map.

Tables: Total Annual Community Living Spending per Medicaid Recipient among People with Disabilities (PWD), all ages, by state (e.g., BEST more annual community living spending and WORST less annual community living spending)

BEST	
State	**Spending**
New York	$24,352,337,631
Pennsylvania	$11,501,811,134
California	$8,410,312,977
Texas	$7,618,132,390
Minnesota	$6,173,071,844
Ohio	$5,001,553,920
Massachusetts	$4,556,312,732
Washington	$4,305,820,298
New Jersey	$3,824,623,422
Florida	$3,251,864,825
Virginia	$3,219,386,718
Illinois	$3,058,280,786
Michigan	$3,027,565,788

GOOD	
State	**Spending**
Colorado	$2,754,594,931
Maryland	$2,711,437,156
Missouri	$2,688,024,106
North Carolina	$2,505,258,245
Indiana	$2,208,076,707
Georgia	$2,190,963,186
Connecticut	$2,091,930,016
Arizona	$1,920,769,216
Oregon	$1,775,850,967
Tennessee	$1,562,038,023
Wisconsin	$1,424,775,390

Kentucky	$1,334,581,396
Iowa	$1,197,226,211

FAIR	
State	**Spending**
New Mexico	$1,158,087,103
Louisiana	$1,103,252,098
South Carolina	$1,075,741,097
Maine	$857,414,381
West Virginia	$805,667,743
Kansas	$761,048,229
District of Columbia	$740,371,226
Nebraska	$679,752,487
Oklahoma	$670,926,590
Alabama	$650,206,150
Arkansas	$623,457,863
Mississippi	$585,345,772

WORST	
State	**Spending**
Idaho	$572,632,165
Delaware	$566,244,540
Rhode Island	$534,479,276
New Hampshire	$484,934,732
Nevada	$463,175,032

Utah	$442,841,164
Vermont	$385,922,326
Alaska	$369,377,284
North Dakota	$286,403,120
Montana	$274,181,608
Hawaii	$260,491,300
South Dakota	$195,292,498
Wyoming	$161,492,271

The Americans with Disabilities Act Participation Action Research Consortium (ADA-PARC) is a collaborative national project to document participation disparities experienced by people with disabilities at the national, state and city levels. For more information on your state visit ADAPARC.ORG. For more information about your rights go to the ADA National Network ADATA.ORG. Please contact hammel@uic.edu if you have any questions.

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[1] Office for Civil Rights. (2018, June 28). *Serving people with disabilities in the most integrated setting: Community living and Olmstead*. U.S. Department of Health and Human Services.

[2] "Centers for Medicare & Medicaid Services (CMS) & Mathematica, *Medicaid LTSS Users & Expenditures — 2022 Data Tables*, A2_LTSSExpDlvrySystm_2022.xlsx ('A.2.3 All-HCBS'); see CMS LTSS Reports & Evaluations. Methodology and definitions from Mathematica (2024) *Users & Expenditures by Service Category, 2022* and *Methodology for Identifying Medicaid LTSS Expenditures and Users, 2022*." Medicaid+2Medicaid+2